



MANJU AVINASH WARDEKAR
Nest Bungalow
Mukund Nagar Off Sujay Garden
Pune-4110037
Tel No: 918805821913
PID: 1281226

Ref.:Dr.BIDAYE MEETALI MBBS,MRCP(LONDON)

SID: 126019613

Collection Date:

30-04-2026 11:19 AM

Registration Date:

30-04-2026 11:19 am

Report Date:

30-04-2026 04:15 PM

REPORT

Age:84.10 Years Sex:FEMALE

Complete Blood Count (EDTA Whole Blood)	Result	Biological Reference Interval
Hemoglobin (Hb), EDTA whole blood Method: Photometry	8.00	11.6 - 15.0 g/dL
Total Leucocytes (WBC) count Method : Coulter Principle	8,600	4000-10000/ μ L
Platelet count Method : Coulter Principle	237,000	150000 to 410000 / μ L
Red blood cell (RBC) count Method: Coulter Principle	2.67	3.92 - 5.13 x 10 ⁶ / μ L
PCV (Packed Cell Volume) Method: Calculated	23.20	35.50 - 44.90 %
MCV (Mean Corpuscular Volume) Method: Derived from RBC histogram	86.60	78.2 - 97.9 fL
MCH (Mean Corpuscular Hb) Method: Calculated	29.80	27.0 - 32.0 pgms
MCHC (Mean Corpuscular Hb Conc.) Method: Calculated	34.40	31.5 - 34.5 g/dL
RDW (RBC distribution width) Method: Derived from RBC Histogram	13.20	12.2 - 16.1 %
MPV (Mean Platelet Volume) Method: Derived from Platelet Histogram	7.30	7.9 - 10.8 fL
WBC Differential Count (Method: VCSn / Microscopy / Calculated)		
Neutrophils	79	40 - 80 %
Absolute Neutrophils	6,794	2000 - 7000 / μ L
Eosinophils	1	1 - 6 %
Absolute Eosinophils	86	20 - 500 / μ L
Basophils	0	0 - 2 %
Absolute Basophils	0	0 - 100 / μ L
Lymphocytes	14	20 - 40 %
Absolute Lymphocytes	1,204	1000 - 3000 / μ L
Monocytes	6	2 - 10 %
Absolute Monocytes	516	200 - 1000 / μ L
-	+#	



Awanti Golwilkar Mehendale
Dr.(Mrs.) Awanti Golwilkar Mehendale
MBBS,MD(Path) Regn.No:2000/02/1052
A.G Diagnostics Pvt. Ltd.



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Complete Blood Count Findings

R.B.C. : Mild anisocytosis, mild poikilocytosis, mild polychromasia.

W.B.C. : No abnormality detected

Platelets : Adequate

Remark : ON FOLLOW UP.

.? H/O BLOOD LOSS.

.SUGGESTED CLINICAL CORRELATION, B12, FOLIC ACID SUPPLEMENT & FOLLOW UP.

.
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Mehendale
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Test Description	Observed Value	Biological Reference Interval
Glycated Hemoglobin (HbA1C), by HPLC	7.3	4.0 to 5.6 %
Estimated Average Glucose (eAG)	163	mg/dL

Sample : EDTA Whole Blood

Interpretation :

HbA1C level reflects the mean glucose concentration over previous 8-12 weeks and provides better indication of long term glycemic control.

For diagnosis of Diabetes Mellitus (≥ 18 yrs of age) :

5.7 % - 6.4 % : Increased risk for developing diabetes.

≥ 6.5 % : Diabetes

Therapeutic goals for glycemic control :

Adults : < 7%

Toddlers and Preschoolers : < 8.5% (but > 7.5 %)

School age (6-12 yrs) : < 8%

Adolescents and young adults (13 - 19 yrs) : < 7.5 %

The A1c target should be individualized based on numerous factors, such as age, life expectancy, comorbid conditions, duration of diabetes, risk of hypoglycemia or adverse consequences from hypoglycemia, patient motivation and adherence.

Levels of HbA1C may be low as result of shortened RBC life span in case of hemolytic anemia.

Increased HbA1C values may be found in patients with polycythemia or post splenectomy patients.

In patients with Homozygous forms of rare variant Hb(CC,SS,EE,SC), HbA1c cannot be quantitated as there is no HbA. In such circumstances glycemic control needs to be monitored using alternative methods like plasma glucose levels or serum Fructosamine.

Estimated Average Glucose (eAG) :

1. eAG is an estimated average of blood glucose level over previous 8-12 weeks.

2. HbA1C and eAG have a linear relationship.

3. The eAG is not a substitute for fasting and post prandial blood sugar measurements as prescribed by your physician or home blood glucose monitoring.

Ref : American Diabetes Association (Standards of Medical Care in Diabetes - 2025)



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Test Description

Observed Value

Biological Reference Interval

Haematology :

Erythrocyte Sedimentation Rate, EDTA Whole Blood **25**

Female under 50 Yrs : Upto 20mm/hr.
Female 50 - 85 Yrs : Upto 30mm/hr.
Female > 85 yrs : Upto 42mm/hr.
Results corrected to 18 deg. celsius

Technique : Modified Westergren Method, Automated .

1. ESR is markedly elevated in monodonal gammopathy such as multiple myeloma, in severe polyclonal hyperglobulinemia due to inflammatory disease, and in hyperfibrinogenemia.
2. Moderate elevations are common in active inflammatory disease such as rheumatoid arthritis, chronic infections, collagen disease and neoplastic disease
3. ESR has little diagnostic value in these disorders but can be useful in monitoring disease activity.
4. Useful in the diagnosis and in monitoring polymyalgia rheumatica and temporal arteritis.
5. Moderate increase is seen in pregnancy (beginning at the 10th to 12th week) and returns to normal about 1 month postpartum .
6. Red cells with an abnormal or irregular shape, such as sickle cells or spherocytes, hinder rouleaux formation and lower the ESR.

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Carrying forward
Dr. Ajit Golwilkar's
legacy of 5 decades.

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Dr. Golwilkar Awanti
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Dr. Golwilkar Vinanti
MBBS, MD (Pathology)



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Test Description

Plasma Glucose :

Plasma glucose, random by Hexokinase method

Observed Value

209

Biological Reference Interval

< 200 mg/dL

American Diabetes Association
Guidelines 2025

Clinical Chemistry

Urea, serum by GLDH-urease

69

17 to 49 mg/dL

BUN-Blood Urea Nitrogen,serum by calculation

32.24

8 to 23 mg/dL

Creatinine, serum by Jaffe w/o deproteinization

0.96

0.6 to 1.2 mg/dL

Sodium, serum by ISE Indirect

135.00

136 to 145 mmol/Lt

Potassium, serum by ISE Indirect

4.30

3.50 to 5.10 mmol/Lt

Chloride, serum by ISE Indirect

102.00

98 to 107 mmol/Lt

SGPT (ALT), serum by Enzymatic Method

16

7 to 45 U/Lt

SGOT (AST), serum by Enzymatic method

22

>or= 14 years : 8 - 43 U/Lt

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Mehendale

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Urine Routine Examination

(Sample : Urine, Automated / Semiautomated)

Physical

Quantity Examined

5.0

ml

Method : Visual

Appearance

Clear

-

Method : Visual / Automated

Colour

Yellow

-

Method : Visual / Automated

Chemical (Dipstick)

pH

5

4.6 - 8.0

Method : Indicator Principle

Protein

Absent

Absent

Method : Sulphosalicylic Acid/ pH Indicator

Glucose

Present Trace

Absent

Method : GOD-POD / Benedict's

Acetone

Absent

Absent

Method : Sodium Nitroprusside reaction

Bile Pigments

Absent

Absent

Method : Diazo Reaction / Fouchet's test

Urobilinogen

Not significant

Not Significant

Method : Modified Ehrlich / Watson Schwartz

Microscopy / Flow cytometry

R.B.Cs

Absent

0 - 2 per hpf

Pus cells

1-2

0 - 5 per hpf

Epithelial cells

1-2

0 - 5 per hpf

Casts

Not Detected

Not detected

Crystals

Not Detected

Not detected

-

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Home Visit

DONE

End of Report


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